ST VINCENT'S HOSPITAL MELBOURNE

ST. VINCENT'S MELBOURNE

ENDOSCOPY REFERRAL AND CONSENT

Email completed form to Endoscopy.Waitinglist@svhm.org.au or fax to 9231 4132 if not in specialist clinics

UR No.: _____

Surname: _____

Given Name: _____

D.O.B.: _____

Please fill in if no Patient Label available

SOURCE: ☐ Clinic (state) _____ ☐ Ward ☒ Rooms – also forward referral letter ☐ Other _____

REFERRING DR Chamara Basnayake Unit _____

Signature _____ ph/pgr _____ Date _____

ADDITIONAL PATIENT DETAILS

☐ VRE / CRE ☐ Adverse Reactions ☐ Other _____ ☐ INTERPRETER required language _____

FOLLOW UP PLAN Clinic _____ ☐ GP ☐ Private rooms

CATEGORY ☐ 1 Urgent (within 30 days) ☐ 2 Semi-Urgent (within 60 days) ☐ 3 Non-Urgent (within 180 days)

☐ **REPEAT** timeframe required { } weeks or { } months or ☐ 12 month repeat

ADMISSION ☒ Outpatient ☐ Ward (medical reason) _____ ☐ Pre-procedure ☐ Post-procedure

☐ **GASTROSCOPY**

☐ GS3 Upper GI unit list required (GS3.Waitinglist@svhm.org.au)

Indication _____

- ☐ Bleeding / Hematemesis / Melena
- ☐ Iron deficient
- ☐ Iron deficient anaemia
- ☐ Dysphagia
- ☐ Varices
- ☐ Weight loss
- ☐ Abnormal imaging (attach report)
- ☐ Gastric ulcer review
- ☐ Pain _____
- ☐ Nausea/Vomiting
- ☐ Reflux symptoms
- ☐ Small bowel biopsy (coeliac screening)
- ☐ Barrett's Screening or surveillance
- ☐ Pre-Bariatric Surgery (GS3 list)
  nutritional bloods required at time of procedure
- ☐ Other _____

☒ **COLONOSCOPY** ☐ **FLEXIBLE SIGMOIDOSCOPY**

☐ GS1 colorectal unit list required (Colorectal.Waitinglist@svhm.org.au)

Indication _____

- ☐ PR bleeding
- ☐ Iron deficient
- ☐ Iron deficient anaemia
- ☐ Known large polyp (Attach report)
- ☐ Abnormal imaging (Attach report)
- ☐ Weight loss % of body weight loss _____ duration _____
- ☐ Abdominal pain
- ☐ IBD assessment ☐ IBD surveillance
- ☐ Haemorrhoidal banding
- ☐ Altered bowel habit ☐ diarrhoea ☐ constipation duration _____
- ☐ Surveillance ☐ previous Cancer ☐ family history Cancer ☐ previous polyp
- ☐ Other _____

PREPARATION *Outpatient bowel preparation only. For inpatient bowel preparation refer to inpatient bowel preparation guidelines*

☐ **Standard Prep** ☐ **3L Glycoprep** ☐ **Fleet enema (on arrival)**

☒ Other **extended**

INTERVENTIONAL REFERRALS Complex therapeutic procedures must be approved by Registrar, Fellow, Consultant (state below)

Interventional Dr required _____ Approved by _____ Unit _____

- ☐ ERCP
- ☐ PEG
- ☐ HALO
- ☐ POEM
- ☐ EMR ☐ Upper ☐ GI Colonic
- ☐ Diagnostic EUS
- ☐ Interventional EUS
- ☐ ESD
- ☐ BALLOON ENTEROSCOPY ☐ antegrade ☐ retrograde

Other _____

☐ CAPSULE ENDOSCOPY (not waitlisted – book thru DPU)

Anaesthetics – discussion with clinical lead criteria

- End stage lung disease (e.g. on home oxygen)
- ICU admission/cardiac event in previous 3 months
- Unstable cardiac disease
- Severe pulmonary hypertension
- Previously diagnosed with Covid 19
- History of significant anaesthesia complications or difficulties e.g. failed intubation, malignant hyperthermia

☐ **YES – Contact required with anaesthetic clinical lead**

Patients requiring anaesthetic review are unsuitable for SVOP

Recommendations _____

All referrals to anaesthetic clinic are to be discussed with the anaesthetic clinical lead for endoscopy on 9231 4471

CO-MORBIDITIES

- ☐ BMI over 50
- ☐ Severe Cardiac Disease _____ ☐ PPM ☐ AICD
- ☐ Diabetes – medication plan discussed with patient and recorded in MRO ☐ Diet ☐ Oral hypoglycaemic agents ☐ SGLT2 Yes / No ☐ Insulin
- ☐ Renal _____
- ☐ Severe Respiratory Disease _____
- ☐ History of Covid-19. Date of diagnosis _____
- ☐ Other _____

St. Vincent's On The Park – see criteria back page

criteria reviewed and suitable? ☒ Yes ☐ No

ANTICOAGULANTS AND PLAN

☐ **NONE** No change Stop **before** endoscopy

☐ Aspirin/Asasantin	☐	or stop _____	days before
☐ Enoxaparin/Heparin	☐	or stop _____	days before
☐ Anti-platelet agent	☐	or stop _____	days before
☐ Warfarin	☐	or stop _____	days before
☐ NOAC	☐	or stop _____	days before

☐ Other _____

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